



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : مروه مامون سيف اليزل محمد

المحافظة : القاهرة الحي / المدينة : عين شمس العنوان : =

رقم البطاقة : 28303240100981 رقم التليفون : 01003563675 تاريخ تسجيل الإستمارة : 26-09-2026

العمر (تاريخ الميلاد) : 24-03-1983 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مروه مامون سيف اليزل محمد National ID: 28303240100981	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
 زور ساعد

ID / ١١٤٠٠٢٢٤٠١٠٠٩١١

١٠٠٦٠٢٢٤٠١٠٠٩١١



وزارة الصحة والسكان
للمراكز الطبية المتخصصة
أورام دار السلام
شمال، سابقاً

Name: عرونة مأمون سيف الدين

Age: 38 y

No.: 0044

c.c.

MDT FORM

Clinical data :

IDC
II

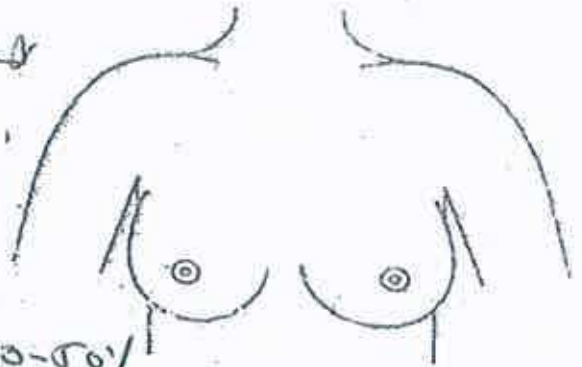
premenopausal Left Breast

Cancer

CT₃, N₁, Mo

ER+ve 7/8, PR+ve 7/8

Her2 -ve, Ki67 40-50%



pt received 4 AC, 4 taxotere

last on 19/5/21

Reason of discussion:

• IE: LTV09 - 3x4

LTV09 - 2x1

for MDT

MDT Outcome and Plan

pt ended MDT 19/5 with partial response
for MDT (as per taken MDT discs).

Signature :

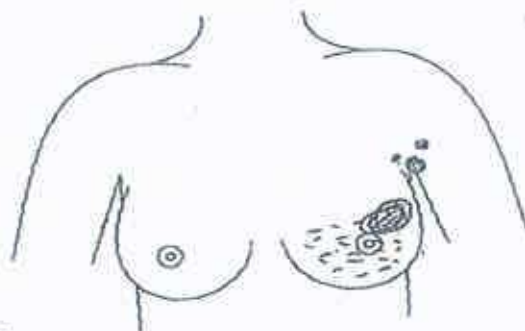


فروہ ماصون سیف
Name: فروہ ماصون سیف
Age: 21
No.: 1127055579

10-12-2020

MDT FORM

Clinical data:



Reason of discussion:

MDT Outcome and Plan:

In view of large lump and mass in upper outer quadrant of right breast (AXY → 4) for MDT.

Signature:

Dr. P.S. CESH requested to assess
Cervix, lateral breast.
(Under breast)
Cervix, lateral breast free for



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m6493

Female	النوع	مروه مامون سيف	يض
القاهرة	العنوان	37 Y	37 Y
30/11/2020 15:21	تاريخ استلام العينة		تاريخ العينة
02/12/2020 14:07	تاريخ التقرير	مركز اورام دار السلام	تحليل
		مركز اورام دار السلام	

Primary Sample

Paraffin Block

Test

ER & PR & HER2 neu & Ki67 (ER & PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment

Technique

sample recieved was treated by : CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti progesterone receptor (PR) (1E2) Rabbit monoclonal primary antibody.

Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Sample recieved was treated by : - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result

ER: moderate nuclear staining of 80 % of tumour cells

PR: moderate nuclear staining of 30 % of tumour cells

HER2/neu protein over expression : No membrane staining of tumor cells.

Ki67: positive nuclear staining of 40-50 % of tumour cells

Referral Lab

Comment

Conclusion

ER: POSITIVE (SCORE 7/8)

PR: POSITIVE (SCORE 5/8)

HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE

#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS 40-50% (CORE BIOPSY)

Pathologist:

د/أينس رمزي

Lab Director: د/ تيريزا ماهر

13 El-sheikh Rehan Street, Abeldein, Cairo

Tel: (155) 7948544 - 7950096 Fax: (202) 7948544

Mobile: 01144297292

website: <http://www.healthlabs.net>

email: pathologychl@yahoo.com

facebook: قسم الباثولوجي - الادارة المركزية للمعمل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Immunohistochemistry

[DOCTYPE: Page 1 of 1



Patient Name: Marwa Maamoun Seif Al Yazal Mohamed	Date : 26/10/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID: 5544/2020	Pat. ID: 22650

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM i

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Youmna tarek

26/10/2024 12:07 PM



19911

@f alborgscan.com



Visit Number: 61124530774
Patient Name: مروه مأمون سيف بزن
Age / Sex: 42 year / Female
Patient ID: 2136257049

Client Name: د. محمد أحمد مأمون
Referred by Prof: احمد لوفل
Exam Date: 23 Nov 2024
Printed: 11/30/2024 2:15:07 PM

NM

TC-99M MDP FOLLOW UP SKELETAL SCINTIGRAPHY

HISTORY

- 40-year-old lady, presented with history of breast cancer. She is referred for follow up.

TECHNIQUE

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

FINDINGS

- Compared to the last bone scan dated 24/12/2020, the current scan revealed similar unchanged scintigraphic features with no evidences of osseous tracer uptake abnormalities that would account for newly developing bone secondaries.
- Exaggerated Degenerative features are depicted at multiple levels of the spine and big joints. All showed stationary course with time.

IMPRESSION

- The skeleton is keeping free from osseous metastases.

Dr. Sherif Elfateh, M.Sc.

Dr. Abu-Bakr Elhawary, MD
Nuclear Medicine Consultant



CamScanner الممسوحة ضوئياً بـ



Patient Name: Marwa Maamoun Seif Al Yazal Mohamed	Date : 12/10/2024
Exam : C.T CHEST	Age : 41 Year(s)
Medical ID: 5544/2020	Pat. ID: 22650

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

- Reported case of left breast cancer underwent mastectomy and received radio, chemo and hormonal therapy.

I. Chest:-

- Evidence of left mastectomy with clear operative bed.
- Left apical, Lingular and middle lung lobar atelectasis likely post radiation pneumonitis.
- Bilateral few subcentimeteric pulmonary nodules, largest at right lower lung lobe apical segment measuring about 5 mm.
- Otherwise No pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal pathologically enlarged lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- Mildly enlarged liver showing homogenous CT texture. No definite focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Apart from left renal upper pole hyperdense lesion measuring about 1x1.1 cm ? hemorrhagic cyst otherwise for further assessment.
- Normal appearance of the pancreas, and both suprarenals (apart from small spenule).
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD is noted otherwise unremarkable appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.



Patient Name: Marwa Maamoun Seif Al Yazal Mohamed	Date : 12/10/2024
Exam : C.T CHEST	Age : 41 Year(s)
Medical ID: 5544/2020	Pat. ID: 22650

Opinion:

- ✦ Left breast cancer showing;
- Left mastectomy with clear operative bed.
- Few bilateral subcentimeteric pulmonary nodules for follow up.
- No evidence of hepatic (on non contrast basis) or osseous metastatic deposits.

Dr. Shaimaa Khaled, MSc.

Signature Prof. Menna Tallah Hanafy

26/10/2024 12:16

Patient's Name: Mawra Maemoun Sif Al-Eizel Mohammed
Date: 2026-08-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent left MRM.

I- CHEST:

- Left upper lobe subpleural reticulations likely post radiation sequelae.
- Few tiny bilateral scattered pulmonary nodules measuring 2-3 mm in diameter.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mildly enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ Few tiny bilateral scattered pulmonary nodules likely inflammatory for follow up.
- ❖ No pulmonary or hepatic metastatic lesions.

Dr.sherif Wagdy

Patient's Name: Mawra Maemoun Sif Al-Eizel Mohammed
Date: 2026-08-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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Dr.sherif Wagdy